

Violence & Suicide

11/14/2025

0800-1000

LIVE ZOOM

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Objectives

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This is a **2 hour lecture**. At the end of the first hour on **violence**, students will:

1. Understand the meaning of dangerousness
2. Understand the difference between predicting and assessing for violence
3. Be familiar with a violence risk assessment
 - ▶ Understand the importance of magnitude, imminence, likelihood and frequency in an assessment.
4. Understand the public health problem we face in dealing with violence in US and other countries.
 - ▶ Know the murder rate in the U.S as compared to other countries.
5. Understand the basic cause for child violence in U.S.
 - ▶ Understand role weapons play in violence

Objectives (continued)

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At the end of the **first** hour on **violence**, students will:

6. Be able to discuss the 3 basic threats and what steps should be taken if threats occur in the workplace
 - ▶ Understand myths about violence
 - ▶ Understand who are typically the perpetrators of violence in the workplace
7. Understand the psychiatric diagnoses most associated with violence within the past year
8. Understand key factors to look for in assessing for violence

Violence Objectives (continued)

At the end of this 1 hour lecture on **violence**, students will:

9. Be familiar with demographic factors for violence
10. Become more familiar with Tarasoff warnings and when they apply.
11. Understand the role of elderly individuals in violence.
12. Understand dynamic and static risk factors for violence

Suicide Objectives

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At the end of this 1 hour lecture on suicide, students will:

1. Have a grounding in basic suicide epidemiology
2. Understand the basics of a suicide risk assessment
 - a. Understand Risk factors for suicide
 - b. Understand Protective Factors
 - ◆ Learn Static Risk Factors
 - ◆ Learn Dynamic Risk Factors
3. Recognize high risk factors associated with suicide
4. Understand Suicidal Ideation and Process

Suicide Objectives (continued)

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At the end of this 1 hour lecture on suicide, students will:

5. Recognize risk of a past suicide effort
6. Understand how to assess lethality of intent
7. Understand what is the reaction to surviving
8. Understand the role of a therapeutic alliance
9. Understand the "value" of a suicide contract
10. Understand role of a psychiatric diagnosis in suicide
11. Understand more about suicide notes
12. Learn the major risk factors ("SAD PERSONS") to assess for suicide
13. Understand the meaning of parasuicide
14. Be able to discuss the major points regarding suicide.

Note: This lecture and or the video(s) that accompany it contain content that may elicit uncomfortable feelings in some students.

Assessment of Violence

- ▶ At some point virtually all clinicians will have to deal with a violent patient
- ▶ May have to determine if person is at greater risk for violence, i.e. danger to self or others due to mental disorder (5150)
- ▶ May have to determine if need to protect third parties (Tarasoff)
- ▶ Those who pursue careers in psychiatry and ER medicine will likely see it more often.
- ▶ EX: Agitated and out of control manic patient
- ▶ Patient who has cut his/her wrists and wants to die,
- ▶ Patient on methamphetamines who is threatening hospital staff.

Assessment of Violence

- ▶ **Violent Crimes:**
 - ▶ Assault/Homicide
 - ▶ Robbery
 - ▶ Rape
 - ▶ Domestic Violence
 - ▶ Workplace Violence
 - ▶ Torture

Assessment of Violence

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- ▶ Dangerousness is not a clinical term
- ▶ Dangerousness is legal term related to:
 - ▶ Propensity to engage in aggressive acts
 - ▶ Level of dangerous behavior must rise high enough to justify involuntary containment

Assessment of Violence

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- ▶ Public expects mental health professionals to be especially skilled in predicting violent behavior.
- ▶ Actually psychiatrists are no more skilled than laypersons in assessing for Long-Term violence.
- ▶ Assessments of risk beyond short-term are less accurate (1-2 days)
- ▶ Mental health professional **ARE** in a position to assess violence in clinical settings.
- ▶ Risk assessments, like weather forecasts, need to be updated frequently.

So, How Can We Assess Potentially Violent Individuals?

**Perhaps from a trusted
fortune teller?**



**Maybe we can
simply flip a coin**



Violence

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- ▶ To decide what to do we must perform:
- ▶ **A RISK ASSESSMENT**
- ▶ **NOT try to PREDICT the FUTURE**

Should He Be Involuntarily Hospitalized? Why?



NO way to tell until we assess his risk

**What Should We Assess to
Determine if Someone is
Currently Dangerous?**

Components of Dangerousness

1. Magnitude of harm
2. Imminence of Harm
3. Likelihood harm will take place
4. Frequency of Aggressive behavior

Assessment of Violence

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1. Magnitude:

- ▶ A continuum from:
 - ▶ verbal outbursts,
 - ▶ to pushing,
 - ▶ to slapping
 - ▶ to murder

2. Likelihood

- ▶ Always assessed with Magnitude
 - ▶ Ex: man uses IV amphetamines, in fights daily.
 - ▶ He would be rated as high likelihood of future fights.
- ▶ To make determination of whether to release a person from ER...one must make careful assessment taking magnitude and likelihood into consideration

Assessment of Violence

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3. Imminence:

- ▶ Concerns likelihood of violence in near future
 - ▶ Note: Seclusion & Restraint only used for imminent violent /dangerous behavior

4. Frequency

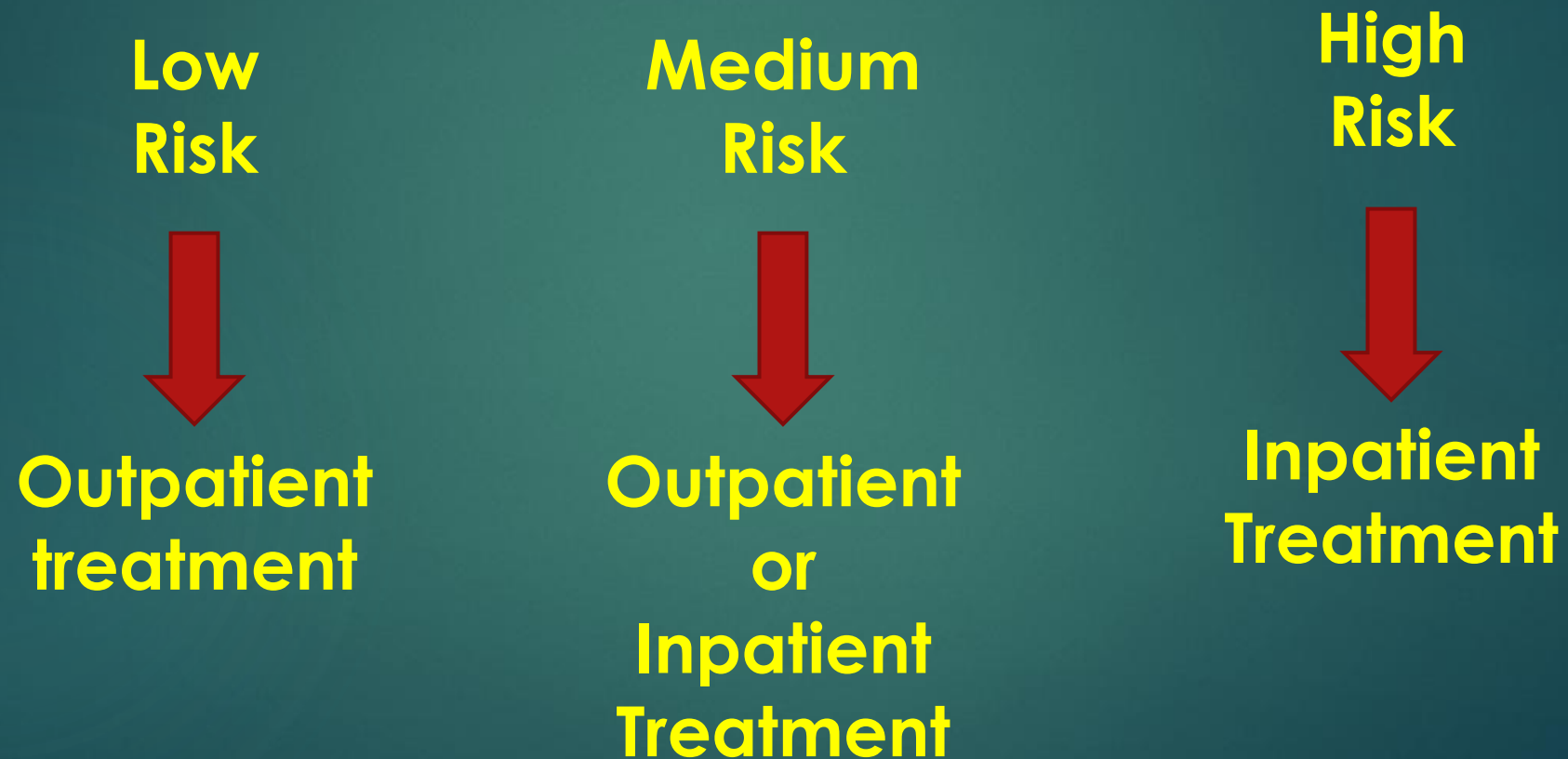
- ▶ Simply because violent behavior may be frequent, that is not key factor
- ▶ An infrequent act may be much more dangerous
 - ▶ Child abuse of only one time

Assessment of Violence

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- ▶ Clinicians must determine whether magnitude + likelihood + imminence + frequency, places an individual in one of following three categories of risk:
- ▶ Low
- ▶ Medium, or
- ▶ High risk for future violence.

Assessment of Violence



Is Violence Really That Big a Public Health Problem?

In 2018,
the US murder rate was
5.0 per 100,000, for a
total of 15,498 murders.
2021: 6.8
2022: 6.1
2023: 5.4

2018

23



CDC 2014 Homicide Data

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- Ages:10 to 24 years old:
 - 86% were killed with a firearm.

CDC 2019-2020 Homicide Data

AGES 0-17

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38,362 Homicides (69% male)

Child homicide rate / 100,000 children has increased annually, ~ 4.3% since 2013

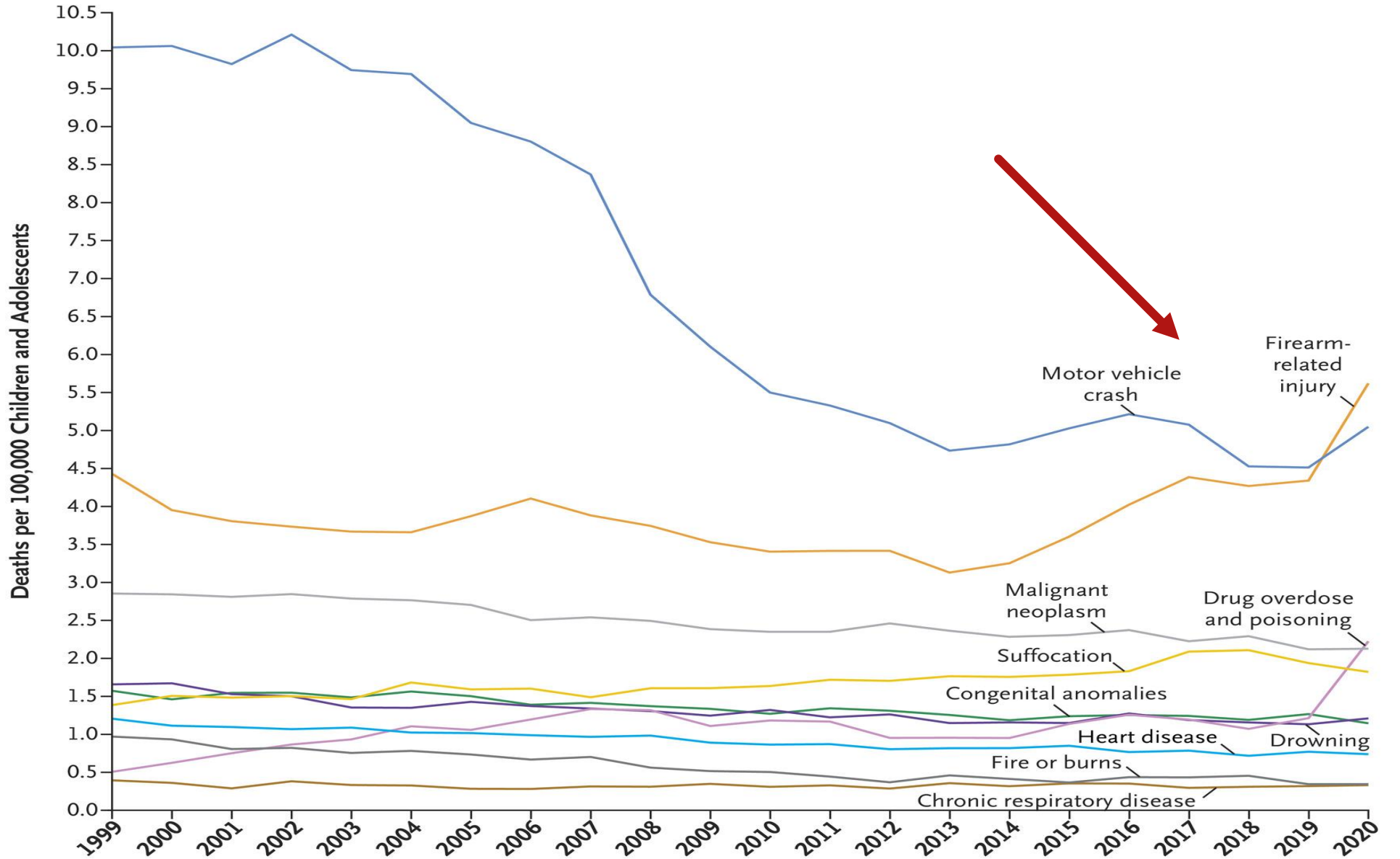
Homicides of children 10 years or younger precipitated by abuse/neglect, by parents/caregivers.

Homicides of 11- to 17-year-olds were most commonly precipitated by crime and arguments and perpetrated by someone known to them, especially friends and acquaintances.

Child and Adolescent Causes of Death 2020

Firearms now in 1st place

New England
Journal of
Medicine



Violence Among Children

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- ▶ From **1985-1995**, the number of **homicides committed by children** has doubled
 - ▶ trend not noted in other countries
- ▶ **Firearm in home is key factor**
- ▶ **1993**: 45% households had firearm
- ▶ **2014**: 32% households had firearm
- ▶ **2024**: 40%, households have a gun.
 - ▶ ~28% more people report having firearms in 2024 than they did twenty years ago.
 - ▶ **Having firearm in home raises risk of family member being killed by factor of 2.7!!!!**

Gun Ownership by the Numbers

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1. 7X Greater likelihood for those living with owners of a handgun to be shot by their spouse or intimate partner, compared with those living in a gun-free household,
2. 6x greater likelihood for those living with handgun owners to be intimidated with a weapon than protected by one.
3. 3X greater likelihood for those with access to firearms to die by **suicide**, compared to those without such access
4. 2X Greater likelihood for those living with handgun owners to die by **homicide**, compared with those living in a gun-free home.

Gun Violence: Children/ Teens

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- ▶ Gun Injuries sent 75,000 children to ER
- ▶ Cost over past 9 years > \$3 billion
- ▶ 11/100,00 children treated in US ERs have gun related injuries (8,300/children each year)
- ▶ ER treatment for gunshot wounds : Males 5x>females
- ▶ Rate highest among males 15-17 years old (86 ER visits /100,000)
- ▶ Cause of wounds:
 - 1. Assault: 49%
 - 2. Unintentional injuries:39%
 - 3. Suicide: 2%



How are these 4 pictures related?

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**Columbine
1999**



**Newtown
2012**

**Parkland Fla.
2018**



**Va
Tech
2007**



Incomplete List of Mass School Shootings

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1. 2007 Virginia Tech: 33 dead, 25 injured
2. 2012 Sandy Hook (Newtown, CT.): 28 dead, 2 injured
3. **2022 Uvalde Texas, 21 dead and 17 injured**
4. 2018 Marjory Stoneman Douglas HS (Fla) 17 Dead, 14 Injured
5. 1966 U. Tx. Austin: 17 dead, 31 injured
6. 1999 Columbine, H.S. (CO) 15 dead 21 injured
7. 2018 Santa Fe H.S.(TX.) 10 dead, 13 injured
8. 2015 Umpqua CC (OR) 10 dead, 9 injured
9. 2005 Red Lake HS (MN) 10 dead, 7 injured
10. 1976 Cal St U, Fullerton, CA 7 dead, 2 injured
11. 2012 Oikos University, Oakland CA 7 dead, 3 injured
12. 1989 Cleveland Element. Sch., Stockton, 6 dead, 29 injured
13. 1991 U. Iowa, (IA) 6 dead, 1 injured

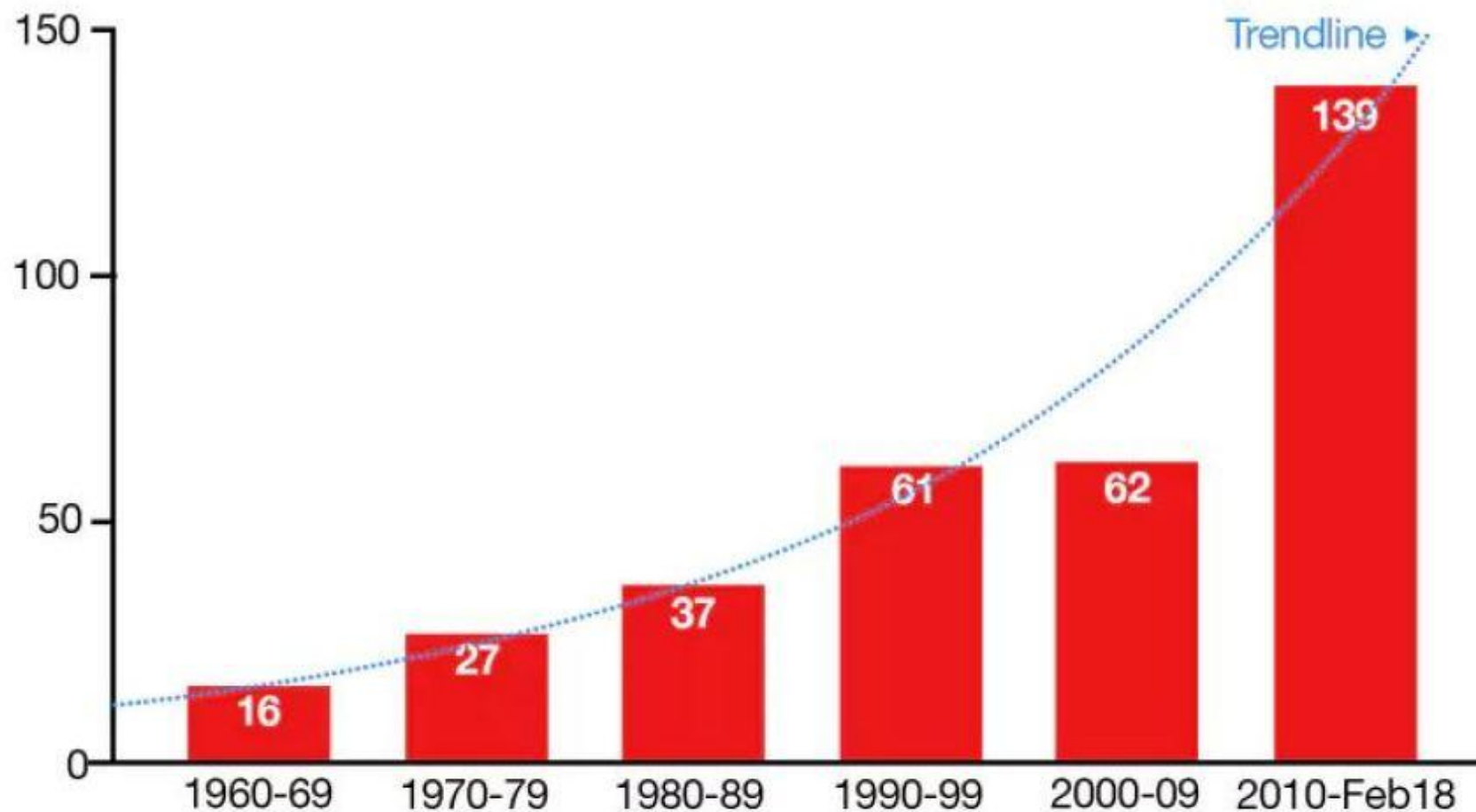
Incomplete Listing of Mass School Shootings

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14. 2006 West Nickel Mines School, (PA), 6 dead, 3 injured
15. 2008 Northern Ill. U. : 6 dead, 21 injured
16. 2013 Santa Monica HS (CA): 6 Dead, 4 Injured
17. 1998 Westside Middle School (AR): 5 dead, 10 injured
18. 2014 Marysville Pilchuck HS. (Wash) 5 dead 1 injured
19. 1992 Lindhurst H.S. Olivehurst , CA 4 dead, 10 injured
20. 2002 U. Arizona Nursing School, 4 dead, 0 injured
21. 1974 Olean HS (NY) 4 dead, 11 injured
22. 1996 Frontier Middle School (UT) 3 dead, 1 injured
23. 1996 San Diego State U. 3 dead, 0 injured
24. 1997 Heath High School, Paducah , KY. 3 dead , 5 injured.
25. ETC, etc.,

Incidence of US school shootings, 1960-2018

Shooting injuries and deaths only, excludes incidental deaths and injuries.
Excludes suicides where no third parties were killed or injured.



School Shootings and Mental Illness

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► What are the facts?

1. Only about 1.3% of youth homicides and injury occur at schools, colleges or Universities
2. There is no known profile that allows early identification of a mass shooter.
3. Only a small percentage of school shooters have a psychotic illness.
4. Mass school shootings are not impulsive acts, but rather the product of careful planning.

► What are some possible interventions?

1. Increase screening and mental health resources for troubled youth in K-12.
 - Likely will not decrease school shootings but will decrease drug and alcohol use, learning disabilities, suicide, and perhaps school violence
2. Develop antibullying programs in grades K-12
3. At all school levels educate that all **threats must be taken seriously** and reported to appropriate authorities.
4. Improve school hardening
5. Develop threat assessments teams.

Assessment of Violence Potential

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- ▶ **Types Of Threats**

- ▶ **Direct**

- ▶ Promise To Harm

- ▶ **Contingent**

- ▶ If you do this, then I will do something

- ▶ **Veiled**

- ▶ You Had Better Watch Your Back

- ▶ Abortion clinics have a way of blowing up....

Violence

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- ▶ Myths About Threats
 - ▶ Veiled Threats Less Serious Than Direct Threats
 - ▶ Risk Of Violence Low If No Threat Has Been Made

WORKPLACE VIOLENCE

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- ▶ **Def.:** Verbal or Physical Assault or any violence in workplace even if source unrelated to work environment
 - Ex: Spouse attacks partner at work
- ▶ Accounts for 15% all violent crimes in US
- ▶ 575,000 workplace crimes (2009)
- ▶ 500 workplace homicides (2009)
- ▶ Weapon is often not used
- ▶ Workplace crime dropped 35% since 2002.

WORKPLACE VIOLENCE

- ▶ Who typically attacks?
 - ▶ Customer.....44%
 - ▶ Stranger.....24%
 - ▶ Coworker.....20%
 - ▶ Boss..... 7%
 - ▶ Former Employees.... 3%
 - ▶ Receive most media attention

WORKPLACE VIOLENCE

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▶ Workplace Homicide

- ▶ # 1 cause of occupational death for women
- ▶ # 3 most common cause of occupational death
 - ▶ Motor vehicle accidents (1)
 - ▶ Machinery accidents (2)

▶ Other :

- ▶ About **50% all health care workers** are assaulted in their career
- ▶ 20% teaching hospitals report weapon threat at least monthly
- ▶ **No Tolerance Policy Vital**

WORKPLACE VIOLENCE

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► Workplace Violence

- **Most Dangerous Occupations (hi to low)**
- 1. Bartender (80/1000)
- 2. Police Officer (78/1000)
- 3. Security guard (65/1000)
- 4. Correctional Officer (33/1000)
- 5. Gas Station Attendant (30/1000)
- 6. **Mental Health Professionals** (17/1000)

Issues to consider if a Psychiatrist is injured

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- ▶ Studies show from a 5%-48% chance of a psychiatrist being injured, depending on the study.
- ▶ **Ethical issues** to consider after an assault.
 - ▶ Should the psychiatrist continue to treat the patient?
 - ▶ Now, would this patient be treated differently by the psychiatrist and other unit staff members compared to other patients on the unit?
 - ▶ Would decisions regarding medications be influenced consciously or unconsciously due to the assault.
 - ▶ Would the patient's privileges be impacted for longer than necessary?
 - ▶ Would decisions regarding discharge be negatively influenced?
- ▶ In many cases even temporary removal of the patient from the care of the psychiatrist may be necessary, and may in the long run be the best ethical decision.
- ▶ Chance of a Psychiatric Resident being assaulted during training is 40-50%.

Violence In the Past Year

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<u>Psychiatric Diagnosis</u>	<u>%</u>
None	2
Any psychiatric diagnosis	4
Obsessive Compulsive	11
Panic Disorder	12
Major Depression	12
Mania/Bipolar	11
Schizophrenia	13
Cannabis Abuse / Dependence	19
ETOH Abuse / Dependence	25
Other Drug Abuse / Dependence	35

Violence and Mental Illness

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- ▶ What are the facts?
 - ▶ Violence is common in our society
 - ▶ People fear being the victim of violent crime.
 - ▶ Crime rates have dropped in last 2 decades
 - ▶ **Media has tended to exaggerate link between violence and mental illness.**
 - ▶ This contributes to not only fear felt by many, but also to the stigma experienced by psychiatric patients.

Violence and Mental Illness

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- ▶ **What are the facts?**
- ▶ Only **3-5% of violence** in the U.S can be **attributed to mental illness**.
- ▶ Substance use and certain personality disorders far outweigh role of other mental illnesses,. (e.g., schizophrenia and major depression)
- ▶ In reality, individuals with mental disorders are **far more likely to be victims** than perpetrators of violent crimes.

Violence and Mental Illness

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- ▶ There is a **weak** relationship
- ▶ Most people with mental illness are **not violent**
- ▶ Victims of people with mental disorders are rarely strangers **(Often the mother)**
- ▶ **Substance use disorders** are a much greater risk factor than other mental disorders!!!
- ▶ **Psychotic individuals** are more likely to **commit violent acts** than those who are nonpsychotic.
 - ▶ So we need to **provide treatment** for such individuals!

Violence and Mental Illness

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Mental illness can sometimes be associated with aggressive or violent behavior.

But

people living with a mental illness and receiving effective treatment are no more violent or dangerous than the rest of the population.

People living with a mental illness are more likely to harm themselves – or to be harmed – than they are to hurt other people.

Violence is not a symptom of psychotic illness.

The relationship between mental illness and violence is complex.

Research suggests there is little relationship between mental illness and violence when **substance use** is not involved. (e.g. . Drugs & Alcohol)

Mass Shooting and Mental Illness

Mass Shootings and Mental Illness

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- ▶ What are the facts?
- ▶ 1. Mass shootings by those with serious mental illness is < 1% annual gun related homicides
- ▶ 2. Deaths by suicide using firearms account for the majority of yearly gun related deaths.
- ▶ 3. Overall contribution of those with serious mental illness to violent crimes is about 3%. Of these an even smaller number have used firearms.

Mass Shootings and Mental Illness

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- ▶ What are the facts? (continued)
- ▶ 4. Laws to reduce gun violence that focus on a population representing < 3% all gun violence will produce a low yield.
 - Perpetrators of mass shootings usually do NOT have a history of involuntary psychiatric hospitalization. Thus, using data bases for such individuals produces a low yield.
- ▶ 5. Gun restriction laws that focus on those with mental illness perpetuates:
 - ▶ **Myth** that mental illness leads to violence, and
 - ▶ **Myth** that gun violence and mental illness are strongly linked

Violence and Mental Illness

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- ▶ Patients with neurocognitive disorders (delirium/dementia) are at increased risk for violence.
- ▶ Ex.:
Patient with Alzheimer's dementia strikes out at wife because he thinks his wife had been talking to him from inside a wall in order to confuse him.

Substance Use and Violence Risk

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- ▶ The association between substance use disorders and violent behavior suggests a risk multiplier of 12-16 compared to the risk level of undiagnosed persons.

Substance Use and Violence Risk

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- ▶ **Drugs and alcohol** are strongly associated with violent behavior
- ▶ Majority of persons involved in violent crimes are under the influence of **alcohol** at the time of their aggression
- ▶ > 50% of all violent events, including murders, were preceded by alcohol consumption by the perpetrator of a crime, the victim, or both

Roth 1994

Assessing Risk Factors for Violence

Static Risk Factors for Violence

Factors We Cannot Change

Static Risk Factors for Violence

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Age ≤ 13 increases risk, and good predictor of future delinquency and adult criminal activity.

Younger at first arrest

Having a conduct disorder

▶ Gender: men 10x more risk than female

▶ Socioeconomic Status: Poor raises risk

Examples of Static Risk Factors for Violence (CONTINUED)

- ▶ Childhood Physical and sexual abuse increases risk,
 - serial rapists: studies showed that ~76% had experienced childhood sexual abuse
- ▶ Intelligence and educational level:
 - Lower level increases risk

Examples of Static Risk Factors for Violence (CONTINUED)

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- ▶ Problematic military history:
 - ▶ dishonorable discharges,
 - ▶ weapon and explosive training
- ▶ Work History:
 - ▶ Good work history lowers risk

Other Issues to be Aware of

Violence Toward Physicians

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- ▶ Bureau of labor statistics indicates that serious workplace violence acts occur in health care and social services industries.
- ▶ Patients cause about half of all nonfatal assaults in the workplace
- ▶ Majority assaults are against nursing staff, however, all health care workers are at risk.
- ▶ **Physicians may also experience verbal abuse or threats; destruction of property; or intimidation with a weapon.**

Past History of Violence is the

▶ Single best predictor of
future violent behavior!

Note: This is so important I guarantee it will be on
the next test in one way or another!!!

Dynamic Risk Factors for Violence

Factors We Can Change

Examples of Dynamic Risk Factors for Violence

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- ▶ Major Psychoses
- ▶ Substance use Disorders
- ▶ Depression
- ▶ Post Traumatic Stress Disorder
- ▶ Antisocial, borderline and paranoid personality disorders
- ▶ Employment instability

Examples of Dynamic Risk Factors for Violence (continued)

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- ▶ Certain CNS Illnesses
- ▶ Family Dynamics
- ▶ Weapon availability
- ▶ Victim availability
- ▶ Residential instability
- ▶ Environments with increased social control and/or strain

Major Psychoses

- ▶ Specific Psychotic symptoms associated with enhanced risk include persecutory delusions (inducing perceived threats) and thought insertion/control (inducing perceived loss of internal controls)

Delusions and Violence

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- ▶ **Increased Chance of Violence If:**
 - ▶ Feel mind dominated by forces beyond your control
 - ▶ Feel thoughts put into your head
 - ▶ Feel people want to hurt you
 - ▶ Feel people following you

Substances Associated with Violence

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- ▶ Alcohol
- ▶ Amphetamines and Cocaine
- ▶ Anabolic Steroids
- ▶ Inhalants
- ▶ Phencyclidine (PCP)

CNS Illnesses Associated with Violence

- ▶ Brain tumors
- ▶ HIV Disease
- ▶ Seizure Disorders
- ▶ Traumatic Brain Injury (TBI)

Cognitive Impairment and Violence Risk

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- ▶ Brain injury or illness can result in aggressive behavior
- ▶ After a brain injury, formerly normal individuals may become verbally and physically aggressively

Characteristics of Aggression Associated with Brain Injuries

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- ▶ Reactive behavior triggered by trivial stimuli
- ▶ Lack of planning or reflection
- ▶ Non-purposeful action with no clear aims or goals
- ▶ Explosive outbursts without a gradual buildup
- ▶ **Note:** Violent acts by people with epilepsy are rare

Family Dynamics

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- ▶ Risk for violence toward significant others may be enhanced when those persons are involved in setting limits or are perceived by the individual to be threatening or hostile.

Interviewing The Violent Patient

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- ▶ Be calm and speak softly
- ▶ Questions should be non-judgmental
 - ▶ “You appear upset,; perhaps you can tell me why you feel that way”.
- ▶ Assure escape route
- ▶ Allow personal distance
- ▶ Have as many staff present as needed
- ▶ Individual and clinician should be seated.
- ▶ **Avoid direct eye contact**
- ▶ Project sense of empathy and concern
- ▶ Family, friends or police interviewed
- ▶ Review all available records
 - ▶ Hospital
 - ▶ School
 - ▶ Employee

Key Points Regarding Violence

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- ▶ Always Check Other Sources of Information
 - ▶ Family
 - ▶ Medical Records
 - ▶ School Records
 - ▶ Employers
 - ▶ Police

Preventing Violence

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Restraint and or Seclusion (R&S)

Used only for imminent danger

Patient told he will be placed in (R&S)

Sufficient number of trained staff place person into R&S

Must be on 1:1 observation

Medications can be used to calm patient

Ex: Haloperidol and lorazepam (oral or IM)

S.F. General Psychiatric ER

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Chronicle / Lance Iversen

1. Continuous direct eye contact
2. Notations into record Q15-30 minutes.
3. Patient must be released when no longer considered an imminent danger

Violence

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- ▶ **Tarasoff Warning: CA Supreme Court 1976 and 1982 rulings**
 - ▶ **Duty to Warn and Protect**
 - ▶ **Party Threatened Must be Identifiable / known**

Violence and the Elderly

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- ▶ Elderly patients with **Dementia** may become violent in response to a delusion:
 - ▶ Capgra's Syndrome:
 - ▶ Delusion that family members have been replaced by imposters
 - ▶ Elderly patients with dementia may strike out at care givers for no discernable reason...so be very cautious.

Key Clinical Points Associated With Violence

1. Predicting violence is difficult but often associated with :

- Alcohol or other drug intoxication
- Neurocognitive disorders, such as Alzheimer's disease, dementia, delirium or TBI
- Psychotic Disorders (Especially if untreated)
- Personality disorders i.e., borderline and antisocial
- Prior violent acts
- Stated intention to hurt or kill someone.

Key Clinical Points Associated With Violence

2. When interviewing a potentially violent individual:

- Approach the patient in a slow and tactful manner
- Do not appear threatening,
- Speak softly
- Maintain interpersonal distance
- Stay at same height as the patient.
- Allow for rapid escape; never allow patient between you and the door.
- Use words that **deescalate** (e.g., “I understand you might be thinking of hurting your mother” vs. “I hear you are going to shoot your mother with an AR-15”.)
- Be sure to have sufficient staff present
- Obtain all records to aid in your assessment.
- Be sure to record all your information and your decision in the medical record.

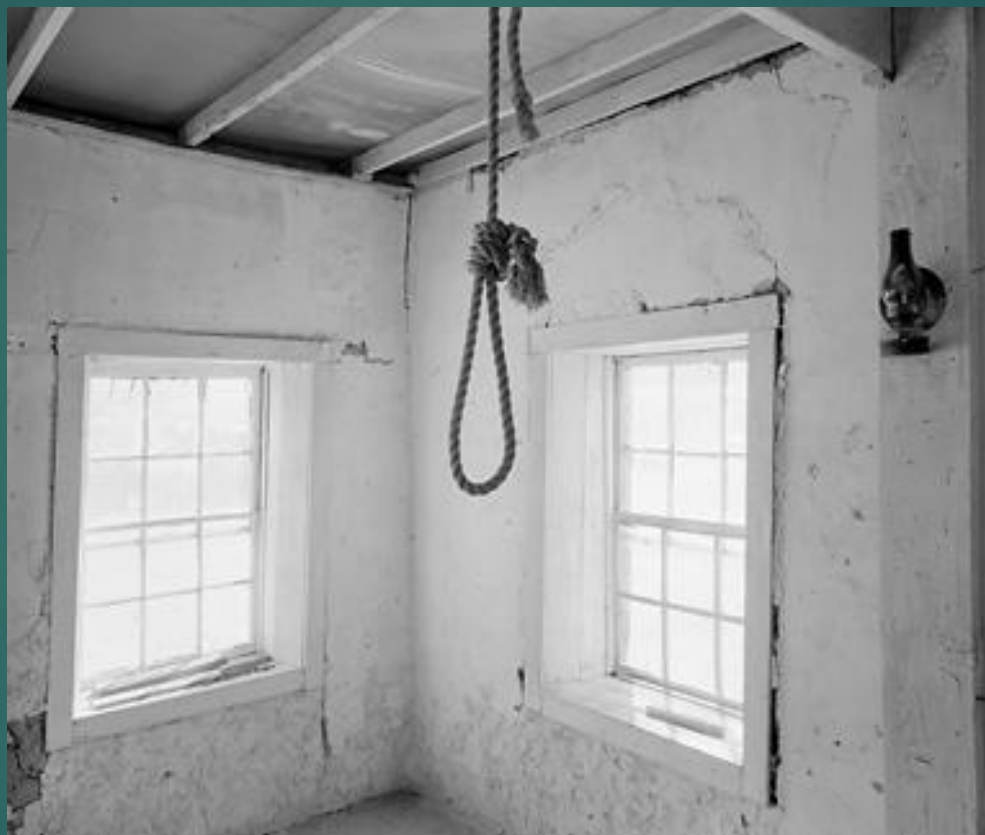
Key Clinical Points Associated With Violence (continued)

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3. Most patients are willing to discuss their feelings
 - Ask what is wrong or why he or she is angry.
4. Violent psychiatric patients for safety, need to be in the hospital
5. Enter Orders in the Medical Record
 - Monitor for violence risk
 - Monitor for assaultive behavior.
 - Document and review plans frequently
6. Treat underlying condition vigorously
7. Outpatients:
 - Monitor risk each session
 - Have patient (or family) remove firearms.
 - Have family call 911 if violence starts

Violence or Suicide?

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Suicide

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- ▶ Definition:
 - ▶ From the Latin for “Self-Murder”
 - ▶ Suicide is the #1 emergency in psychiatry

Vignette

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- ▶ 30 year old physician comes to the ER on her own after overdosing on 25 codeine tablets. She said that she took the pills because her husband of three years had just left her. She also superficially cut her wrists. About 20 minutes later you go into her room in the ER and she says she just wants to go home and go to sleep. She promises she will seek mental health treatment in the morning. She says what she did was stupid and now she says she is thinking clearer.
- ▶ Would you let her go?
 - ▶ If yes why,
 - ▶ If not, why?
- ▶ What would help you to decide what to do next?

Epidemiology

- ▶ A Psychiatrist/physician is not required or expected to predict suicide with precision, but to have carefully evaluated and documented the risk of suicide in his or her patient and to have acted accordingly.

Suicide Epidemiology

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- ▶ Suicide represents a **psychiatric emergency**
- ▶ Yearly in US 650,000 treated in ER after suicide effort
- ▶ **70-90%** committed by people with treatable **psych illness**
 - ▶ Depression
 - ▶ Alcoholism
 - ▶ (So what should this tell us?)

Suicide in the United States

2022 data

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- ▶ 1.6 million people attempted suicide, which means 1 person attempts suicide every 38 seconds.
- ▶ 13.2 million people have serious thoughts of suicide
 - 3.8 million made a plan for suicide
- ▶ **8.6%** of patients admitted to psychiatric unit for suicidal ideation or after a suicide attempt will eventually die by suicide.
- ▶ **>80%** subsequent suicides occurred within a year of initial attempt.

Suicide Epidemiology Methods of Suicide

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<u>Means</u>	<u>Deaths % 2017</u>	<u>Deaths % 2007</u>
1. Firearms	50.6	50.2
2. Hanging	27.7	23.6
3. Poisoning/Overdose	13.9	18.4
4. Jumping	2.4	2.1
5. Cutting/piercing	1.8	1.8
6. Drowning	1.0	1.0

Suicide Epidemiology 2019-2020. (CDC 4/2022)

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1. Heart Disease
2. Cancer
3. COVID -19: ~350,000 deaths
4. Accidents (unintentional injuries)
5. Cerebrovascular disease (stroke)
6. Chronic Obstructive Pulmonary Disease
7. Alzheimer's disease
8. Diabetes
9. Kidney Disease
10. Chronic Liver Disease and cirrhosis
11. Flu/Pneumonia
12. **SUICIDE** (2019) 47,511 (2020) 46,000 13.5/100.000(2021) 47,646=14/100,000,(2022) 49.000=14.2

Note: 17.4 /100,000 during great depression of 1930s (A peak suicide period)

Suicide Epidemiology

- ▶ In a recent study 75% of patients committed suicide within one year of seeing their Primary Care Providers (PCPs) compared to only 1/3 in contact with MH Professionals (MHPs)
- ▶ >2x as many suicide victims had contact with PCPs compared to MHPs in month before suicide (45% vs. 20%)

Suicide Epidemiology

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- ▶ Primary care practitioners are in a better position to intervene to prevent suicide than a mental health professional
 - ▶ 75% of suicide victims see a primary care provider in the year before a suicide
 - ▶ About 38% of these individuals have seen a primary care provider in the month before their suicide
- ▶ Note: Only 20% of suicide victims had seen a mental health professional in the month prior to their death

Suicide Epidemiology

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- ▶ As a Physician, what should you be cognizant of?
 - ▶ Study A.
 - ▶ looked at patients seeing their primary care provider with a complaint of depression and asking for an antidepressant:
 - ▶ **58% were not asked about suicidal ideation**
 - ▶ Study B.
 - ▶ looked at whether suicidal thinking was discussed by a primary care clinician in a last visit prior to suicide in patients who committed suicide within a month of that contact
 - ▶ **62% were not asked about suicidal ideation!!**
- ▶ Studies have shown that only 20% of older adults under treatment in a primary care practice setting will report suicidal thoughts in the course of treatment for depression

Suicide Epidemiology

93

- ▶ Occupational Data

- ▶ Physicians (approx. 400/year)

- ▶ Males: 36/100,000

- ▶ 3 x general population rate

- ▶ Females: 41/100,000

- ▶ 12/100,000 white females > age 25

- ▶ Physicians who commit suicide

- ▶ Depressed & /or on substances

Suicide Epidemiology

94

- ▶ **Occupational Data**
 - ▶ Specialty Risk (in order)
 - ▶ Psychiatrists
 - ▶ Ophthalmologists
 - ▶ Anesthesiologists
 - ▶ Other Jobs at High Risk:
 - ▶ Musicians
 - ▶ Dentists
 - ▶ **Police (2003 report: more die by suicide than by felons)**
 - ▶ Lawyers
 - ▶ Insurance Agents

Suicide Epidemiology

Physician Suicide

95

- ▶ Like other individuals who take their lives, physicians who commit suicide typically have a mental disorder
 - ▶ Depressive disorder
 - ▶ Substance use disorder
- ▶ Female physicians have higher risk of suicide than other women

Suicide Epidemiology

96

▶ OCCUPATIONAL DATA

▶ **Medical School Students:**

- ▶ After accidents, suicide is most common cause of death among medical students

- ▶ 1989-1994:

 - ▶ 15 suicides

 - ▶ 75% in sophomore and junior years

 - ▶ Rate is now lower than previously reported and below national average

 - ▶ May be due to counseling

- ▶ **Female Students:** 3-4 times national average for women of same age group

Suicide

97

- ▶ To decide what to do we must perform:
- ▶ : **A Risk Assessment**
- ▶ **& Not try to Predict the Future**

Suicide

98

- ▶ Problem: Can we determine who will commit / attempt suicide?
 - ▶ **Thinking** of suicide does not equal acting on thoughts
 - ▶ Some have thoughts and:
 - ▶ Never act on them
 - ▶ Act only after days/weeks or years
 - ▶ Some seemingly do it without premeditation
- ▶ How do we deal with **chronic suicide**?
 - ▶ Death thru alcohol or substance abuse
 - ▶ Poor adherence to medical treatment for:
 - ▶ Addiction, hypertension, or obesity

**Before beginning a suicide
assessment we need to know
much more about suicide**

Risk Factors for Suicide

Risk Factors

101

- ▶ **Why study risk factors?**
- ▶ 1. Alerts clinician that there is good reason to suspect patient **MAY** be at higher risk, not that the patient actually is at higher risk
- ▶ 2. They are useful as “**red flags**” that alert a clinician of the need to perform systematic risk assessments for suicide and violence in a patient.

Practical Categorization of Risk Factors

102

► Protective Factors

Examples of Protective Factors Against Suicide

103

- ▶ Strong family and social support network
- ▶ Pregnancy
- ▶ Dependent Children
 - ▶ Except in postpartum psychosis
- ▶ Fear of the actual act of killing oneself
- ▶ Strong religious beliefs against suicide
- ▶ Cultural sanctions against suicide

Protective Factors

104

- ▶ Social support and family connectedness,
 - (Note: family discord increases risk)
- ▶ Parenthood, especially for mothers decreases risk

Protective Factors

105

- ▶ How can you determine quickly about protective factors?
- ▶ Asking, “What would keep you from taking your life?” Specifically pulls for protective factors

Static Risk Factors for Suicide

Factors We Cannot Change

Static Risk Factors

107

- ▶ Age
- ▶ Gender
- ▶ Race
- ▶ Marital Status
- ▶ Socioeconomic status
- ▶ Intelligence
- ▶ Educational Level
- ▶ Family history
- ▶ Personal history

Demographics of Suicide

108

▶ Age

- ▶ Suicide rates increase with age
- ▶ Suicide rate for those >75 is > 3x rate for young persons
- ▶ However, rate among young persons is rising rapidly, especially males 15-24 years old
 - More prone to peer pressure
- ▶ Most suicides (number of cases) now occur among those 15-44.

Demographics of Suicide

109

▶ Gender

- ▶ Men commit suicide 3.7 times as often as women
- ▶ Women attempt suicide 3 times as often as men

Demographics of Suicide

110

- ▶ Men's higher rate of successful suicide is related to the methods they use:
 - ▶ Firearms
 - ▶ Hanging
 - ▶ Jumping
 - ▶ Suicide rate increases with age, peaks > age 75.
- ▶ Women more commonly take an overdose of medication (However, their use of firearms is increasing).
 - ▶ Suicide rate peaks in late 40s-early 50s.

Demographics of Suicide

111

▶ Marital Status

- Marriage reinforced by dependent children significantly lessens the risk of suicide
- Divorce increases risk (3 men /1 woman)

Demographics of Suicide

112

▶ Race

- ▶ White males much more likely to kill themselves than African American males
- ▶ However the suicide rate among African Americans is rising

▶ Religion

- ▶ Historically, suicide rates among Roman Catholic populations have been lower than rates among Protestants and Jews

Demographics of Suicide

113

- ▶ Employment

- ▶ Work, in general protects against suicide

- ▶ Season

- ▶ No significant seasonal correlation with suicide has been found
 - ▶ Contrary to popular belief, suicides do not increase during December and holiday seasons.

Dynamic Risk Factors for Suicide

Factors We Can Change

Dynamic Suicide Risk Factors

115

- ▶ Depression
- ▶ Anxiety
- ▶ Panic Attacks
- ▶ Psychosis
- ▶ Substance Abuse
- ▶ Insomnia
- ▶ Command Hallucinations

Suicide Epidemiology

116

- ▶ **Other risk factors to Be aware of in our assessment**
- ▶ **Health** (50% completers have serious physical illness
 - ▶ (High Rate with: PD, TBI, MS, Cancer, AIDS, Epilepsy)
- ▶ **Family History of suicide**
- ▶ **Access** to firearms,
- ▶ Living **alone**,
- ▶ Loss of loved one,
- ▶ **violence** in past year.
- ▶ **Anniversary reaction**
- ▶ **Release** from psychiatric hospital within last 90 days

Dynamic Risk Factors: Firearms

117

- ▶ Guns in the home substantially increase the risk of suicide in psychiatric patients, as does the recent purchase of a firearm, especially a handgun purchase for women.
- ▶ (So when assessing for suicide you MUST ask about gun ownership! (And access to guns))

Suicide in the United States

118

- ▶ 60% of individuals dying from suicide died on their initial attempt.
- ▶ Scheduling a psychiatric follow-up appointment after an ER visit or an inpatient stay, reduced risk for suicide, even if appointment not kept.

Bostwick Am. J. Psych.
2016

Dynamic Suicide Risk Factors

(cont.)

119

- ▶ Physical Illness
- ▶ Difficult Life Situation

Suicide

120

▶ Physical Health Issues

- ▶ About 1/3 have seen doctor in past 6 months
- ▶ Illness a **factor in about 50%** suicides
- ▶ Chronic Illness Increases suicide risk
 - ▶ Multiple Sclerosis
 - ▶ Head injury
 - ▶ Huntington's Chorea
 - ▶ AIDS
 - ▶ Renal Disease with Dialysis (at high risk)
 - ▶ Mobility Loss/Chronic intractable pain

Suicide Epidemiology

121

▶ Suicide and Mental Health in Elderly

	<u>Mentally Healthy</u>	<u>Mentally Ill</u>
▶ Life not worth living	4.0%	29%
▶ Had “death wishes”	4.0%	28%
▶ Thoughts of Suicide	0.9%	9%
▶ Serious suicidal thoughts	0.0%	2%

Skoog: 1996

High-Risk Factors Associated with Attempted Suicide

122

- ▶ Depression
- ▶ Hopelessness
- ▶ Prior Suicide Attempts
- ▶ Suicidal Ideation
- ▶ Alcohol and drug Abuse
- ▶ Cocaine Abuse
- ▶ Recent loss of an important relationship

Murphy et al., 1992

Important Concept

123

- ▶ Risk factors are **qualitative**, not quantitative measures
 - ▶ They should be considered within the overall context of the clinical presentation.

Campbell 2004

Personality Disorders and Suicide

124

- ▶ Personality disorders associated with depressive symptoms and substance abuse disorders are highly represented among patients who committed suicide

Inpatient Suicide

125

- ▶ 1 500 suicides/year occur in inpatient units
 - ▶ 1/3 of these occur while patient is on 15 minute suicide checks
 - ▶ 75% involve hanging; 20% involve jumping off a roof or out of a window
- ▶ The Joint Commission 2008

Key Concept

126

- ▶ The most dangerous places on the psychiatric unit is the patient's room and the bathroom

**Does it really matter if
someone just thinks about
suicide?**

Suicidal Ideation

128

- ▶ Suicidal Ideation (thoughts) presents in various forms
 - Acute Suicidal Ideation
 - Chronic suicidal Ideation
 - Time based or contingent suicidal threats
 - Combination (e.g., acute intensification superimposed on a chronic suicidal ideation)

Acute Suicidal ideation

129

- ▶ Patients with acute suicidal ideation who express the intent to act on their thoughts should be admitted to a closed psychiatric unit on 1-to-1 observation
- ▶ It is inappropriate to place acutely suicidal patients on 15 minute checks on admission

This is super important!!!!!!!!!!!!!!!!!!!!

Chronic Suicidal Ideation

130

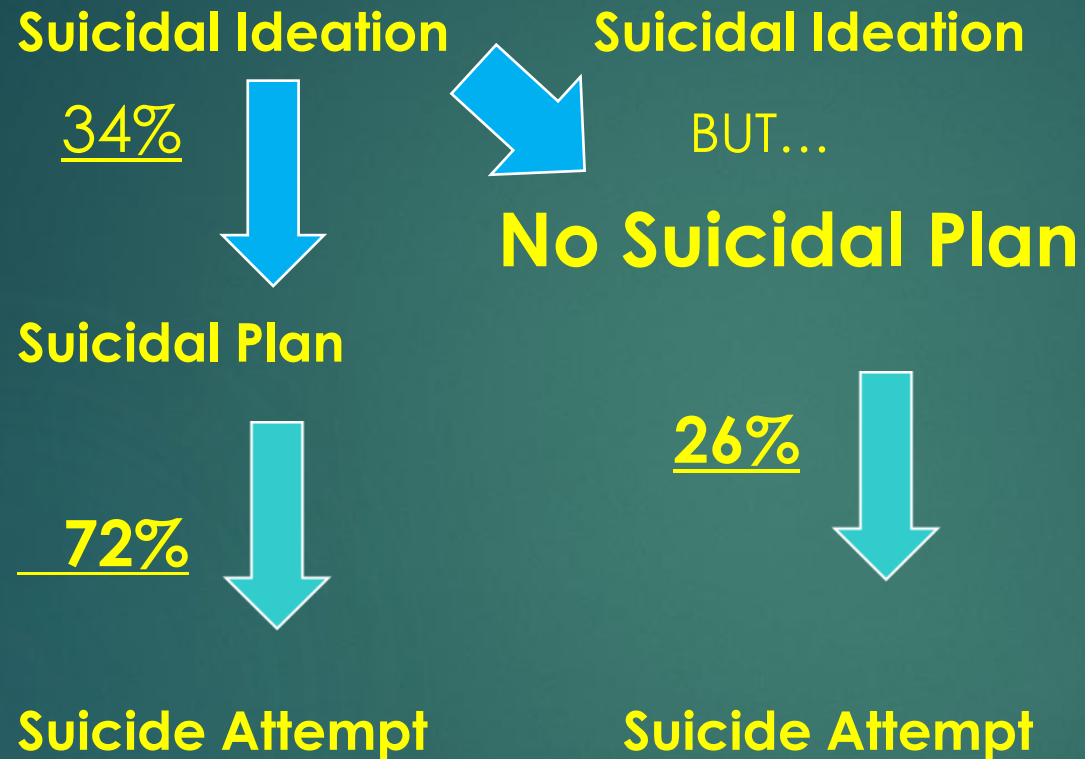
Although actual suicide is a perpetual risk, hospitalization tends to promote regression and should, therefore often be actively resisted...if possible.

- ▶ The patient has a tendency to become progressively dependent on the treatment team and health care system
- ▶ Multiple hospitalizations result in progressive decrements in patient level of functioning

Appelbaum & Gutheil 2007

The Suicidal Process

131



Past History of Suicide Attempts

132

- ▶ A person with a prior suicide attempt is **38 times** more likely to die by suicide than a person who does not have a prior suicide attempt.
- ▶ **Best clinical indicator of a future suicide attempt or completed suicide.**
(Of completers, nearly 30% made prior attempt)
- ▶ Among those who attempt suicide, an estimated **16% will make second attempt within the following year**

Suicidal Intent

133

- ▶ Determining intent in **assessing the lethality of a patient's suicide attempt**
 - ▶ A patient who takes 5 aspirin in the belief that he will die has made a lethal attempt!!
 - ▶ A patient addicted to a drug which he knows will not harm him if he takes additional doses of that drug and takes 5 additional doses has not made a lethal attempt.

Over The Counter Drugs

134

- ▶ Which OTC drug is the most dangerous in a suicide effort? Why?

Reaction of the Patient to Surviving

135

- ▶ An important clinical variable that is easily assessed
 - ▶ Patients who say they wished they had died after a suicide attempt were 2.5 times more likely to eventually commit suicide than those who were glad they survived and those who were ambivalent about the attempt

Henriques 2005

Death Arrangements

136

- ▶ Always inquire about evidence of death arrangements when evaluating a patient who has suicidal ideation, or in the aftermath of an attempt or aborted suicide effort:
 - ▶ Recently completed/updated **will**
 - ▶ Recently purchase **life insurance** (Death benefits may not be payable in the context of suicide)
 - ▶ Written a **suicide note**
 - ▶ Giving away valued **possessions**

The Therapeutic Alliance

137

- ▶ The presence of a therapeutic alliance can be an important protective factor against suicide
 - ▶ It is influenced by a number of factors and can change from session to session
 - ▶ The status of the therapeutic alliance should be assessed regularly and documented

The Therapeutic Alliance

138

- ▶ The absence of a therapeutic alliance with a patient at risk for suicide is a significant risk factor.

Suicide Prevention ("No-Harm") Contracts

Disadvantages of Contracting for Safety

140

- ▶ Barrier to communication
 - ▶ Focus is on contracting, **NOT** on performing a systematic risk assessment
- ▶ Deterrent to Alliance Building
 - ▶ The patient may perceive you as being more interested in “CYA” than helping them
 - ▶ **False Sense of Security**
 - ▶ These “contacts” provide no clinical or legal protection

Key Concepts

141

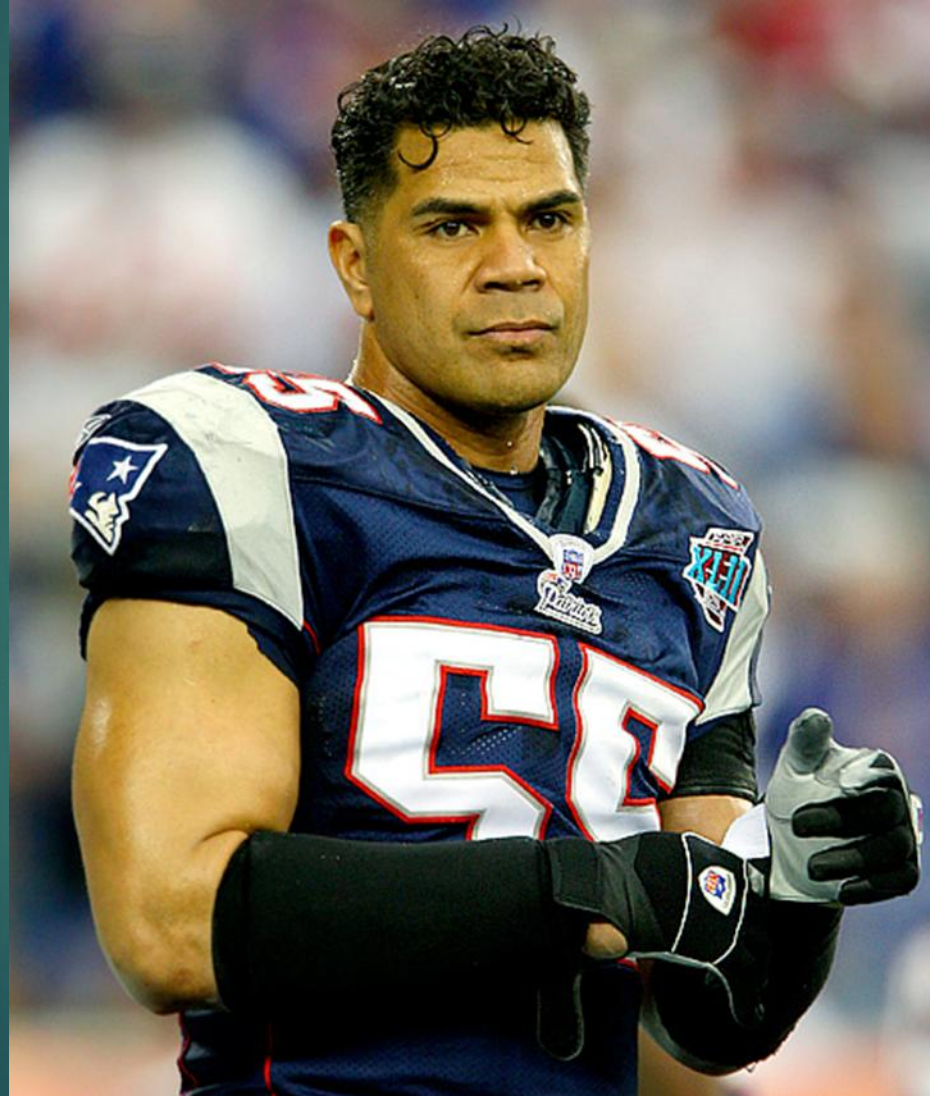
- ▶ Suicide prevention contracts must not take the place of conducting systematic suicide risk assessments
- ▶ Contemporaneous documentation of suicide risk assessments is good clinical care and standard practice.

Famous Individuals Who Have Committed Suicide

Who Is He?

143

Suicide at
age 43 (gun)



**Junior
Seau**

**Chronic
Traumatic Brain
encephalopathy
Due to repeated
head injury**

Ernest Hemingway



Was this a Case of Inevitable Suicide?

144

Hemingway's Suicide Risk Factors

1. Prior Suicide attempts
2. Major depression
3. Cognitive decline
4. Physically ill (cardiac illness)
5. Poor response to: ECT, Antidepressants, Psychotherapy
6. Suicide in other family members
7. Substance Abuse
8. Profound suicidal ideation
9. Weapon possession
10. Severe childhood trauma
11. Multiple head injuries

Suicide

145

- ▶ **95% Suicides Had Psychiatric Diagnosis (Other than Personality disorder)**
 - ▶ Major or other depressive Disorder...75-80%
 - ▶ Schizophrenia.....10-15%
 - ▶ Dementia or delirium.....5%
 - ▶ Alcohol or Substance Use.....15-25%
- ▶ **KEY: > 99% Of People who are Clinically Depressed Do Not Commit Suicide.**

Suicide Notes

146

- ▶ 16% leave a note prior to suicide

- ▶ Note 2/65

To my darling wife I leave you all my worldly goods. I am sorry, dear, but with all my other troubles I fear that I have cancer of the kidneys. I have been passing blood in my urine for some time.

Forgive me.

A business man had not been diagnosed with cancer or any disease, but felt he had the disease. He shot himself with his own weapon. He did not have cancer

Suicide Risk Assessment

147



Suicide Risk Assessment

148

Bill Clinton: Don't Ask Don't Tell Policy

Dr. Zwerin: "If you do not ask about suicide they will NOT tell".

Suicide

149

- ▶ **SAD PERSONS Algorithm** for risk factors
 - ▶ **Sex** Men are more likely to commit suicide than women.
 - ▶ **Age 15-24; elderly**
 - ▶ **Depression: rises risk, hopelessness**
 - ▶ **Previous attempt** majority completed suicides preceded by a prior attempt.
 - ▶ **Ethanol abuse, drugs or alcohol increases risk**
 - ▶ **Rational thinking Loss**
 - ▶ **Social support lacking**
 - ▶ **Organized plan**
 - ▶ **No spouse**
 - ▶ **Sickness**

(Patterson et al, 1983)

Key Concepts

150

- ▶ A suicide risk assessment drives treatment and management of patients at risk for suicide
- ▶ Treatable and modifiable acute suicide risk factors should be identified early and treated aggressively

Suicide Assessment

151

- ▶ **Key Major Risk Factors to assess For:**
 - ▶ Delusional depression
 - ▶ Recovery from a depression (very key)
 - ▶ Race: White males > Black Males
 - ▶ Poor Health
 - ▶ Social ties
 - ▶ Risk Inc. 5X if living alone
 - ▶ Lethality of Plan
 - ▶ Gun: Hi risk
 - ▶ Starvation: Low risk

Suicide Risk Assessment

152

- ▶ **Major Risk Factors** (CONTINUED)
 - ▶ Pervasive **hopelessness**
 - ▶ Family history of suicide
 - ▶ Recent losses/mourning
 - ▶ Failed out of school
 - ▶ **Violence/rage**
 - ▶ Prior Psychiatric Inpatient treatment
 - ▶ Unemployed
 - ▶ Probability of Rescue Small

SUICIDE

153

- ▶ Risk of successful attempt increases:
 - ▶ with prior attempts if < 2 years ago
 - ▶ 1ST 6 Months high risk; 1st 3 months highest risk
 - ▶ 40% depressed individuals who made prior attempt will later commit suicide.
- ▶ How to Decide What to do!
 - ▶ Sound Clinical Judgment must be used
 - ▶ **Document** your interview completely
 - ▶ Consultation with psychiatrist/psychologist
 - ▶ Hospitalize if suicide Risk is High
 - ▶ Low Risk –Outpatient Therapy
 - ▶ Can't hospitalize everyone

Suicide Assessment

154

- ▶ Risk/Rescue Analysis
 - ▶ High Risk/High Rescue
 - ▶ Take overdose of cardiac medication at a party and immediately tell friends
 - ▶ Low Risk/High Rescue
 - ▶ Superficially cut wrist in front of roommate
 - ▶ High Risk/Low Rescue
 - ▶ Take overdose of alcohol & pills and hang self in motel room with door locked
 - ▶ Officer & a Gentleman
 - ▶ Low Risk/Low Rescue
 - ▶ Plan to take overdose of vitamins and not tell anyone

RISK RESCUE ANALYSIS

155

	<u>Risk</u>	<u>Rescue</u>
Jump off GG Bridge (tell no one)	HIGH	LOW
Plan to jump off GG bridge Tell roommate who calls 911	HIGH	HIGH
Superficially cut wrists in front of parents	LOW	HIGH
Swallow entire bottle of cardiac heart meds and drink alcohol (tell no one)	HIGH	LOW
Take 3 extra vitamins tell no one	LOW	LOW

Suicide Assessment

156

Risk Rescue Analysis(continued)

- ▶ By itself, the risk rescue analysis is NOT a predictive instrument
- ▶ Considered along with other factors, such as explicit intention to die, the risk-rescue analysis can be used to assist in suicide risk assessment

Weisman and Wordin, 1972

Suicide

157

▶ Parasuicide

- Definition

- ▶ Self-destructive & nonfatal suicide attempts
- ▶ Usually know means are non-lethal
- ▶ 1/23 complete suicide
- ▶ Most attempts occur with those under 35
- ▶ Most success for those over age 65
- ▶ Women 3x> than Men

Suicide

158

- ▶ Parasuicide (continued)
 - The view that parasuicide and suicide involve totally different populations has been found to be inaccurate.
 - parasuicide is high also in retrospective systematic interview studies of suicide victims.
 - ▶ In a study of young adults, previous parasuicide was found in 60% of young men and 80% of young women.
 - ▶ This a higher rate than among adults in general.
 - ▶ Repeated parasuicide is common in people who commit suicide.
 - ▶ Three or more parasuicides occurred in 17% of men and 56% of women.

Suicide

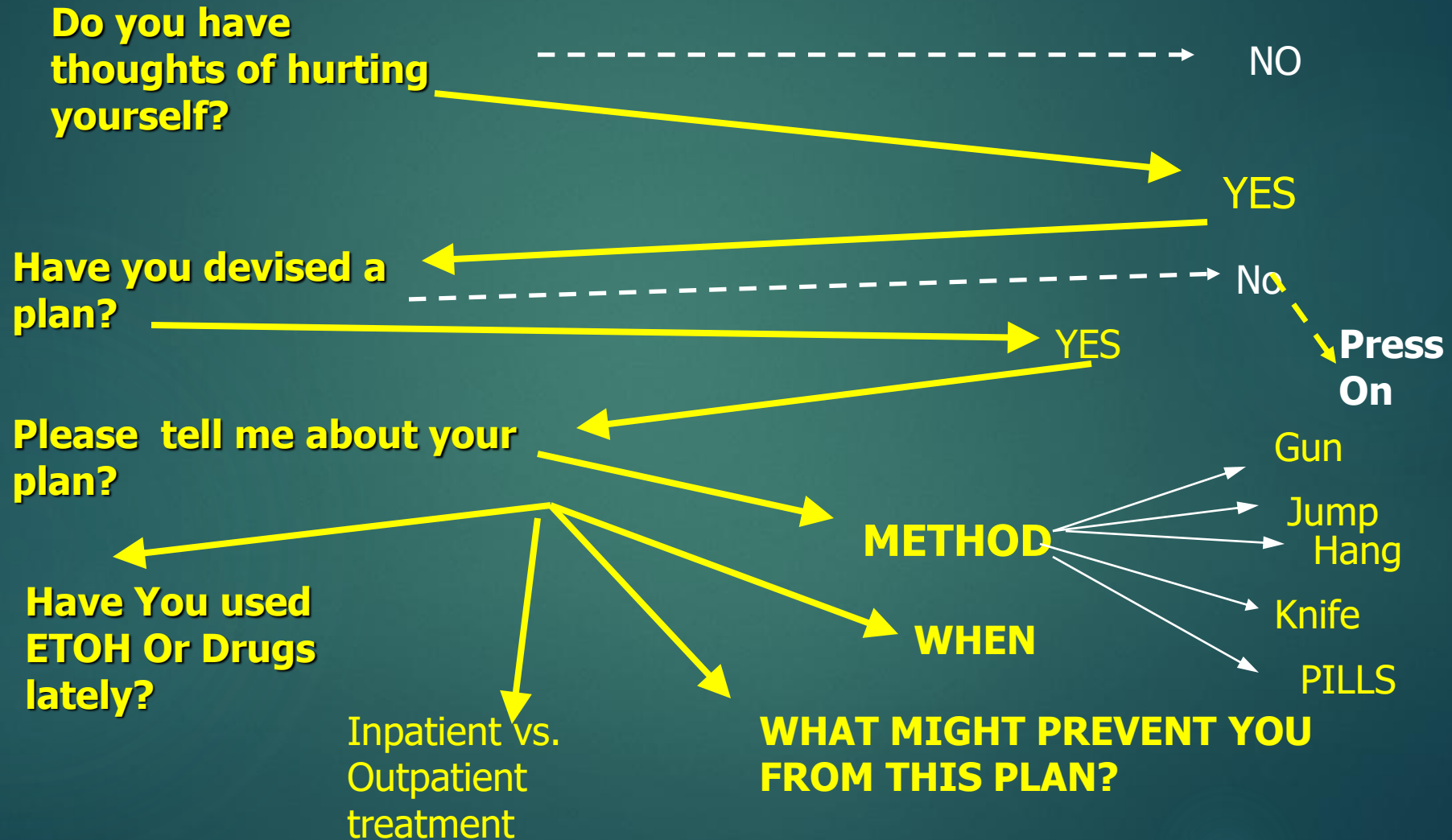
159

- ▶ Parasuicide (CONTINUED)
- ▶ You **MUST** pay attention to previous parasuicide efforts in the assessment of a patient in the emergency department.
 - ▶ Such suicide efforts may indicate a serious risk EVEN if the act was committed several years ago!!
 - ✓ Can we rely on the answers that patients give when we question them about suicidal ideation in emergencies?
 - Answer: An empathic interview with the patient yields an honest answer in MOST instances.

▶

Assessment of Potentially Suicidal Patients

160



What is # 1 suicide site in world?



Stainless steel net installed in 2024. (\$225, 000,000.)
Hopefully will mitigate future attempts.

~2000 suicides since 1937

Key Points Regarding Suicide

162

- ▶ Depressed patients must always be asked about suicidal thoughts
 - ▶ **Asking about suicide does not plant ideas that were not previously there.**
- ▶ Depressed patients must be reassessed for suicidality at each visit.
- ▶ Be sure to perform a suicide risk assessment (SRA)
- ▶ Some suicidal patients should be hospitalized, even if against their will

Key Points Regarding Suicide (continued)

163

- ▶ US suicide rate for men is 4x rate for women
- ▶ Majority of US suicides involve a firearm
- ▶ Over 75% of patients who suicide had contact with their PCP in past year
- ▶ Most patients will disclose suicidal thoughts to PCP if asked.

Key Points Regarding Suicide

164

- ▶ Patients without suicidal plans may be able to be managed in out-patient setting, especially with good support system.
 - ▶ Assess such patients frequently
 - ▶ Prescribe SSRIs with high therapeutic index
 - ▶ Family should remove firearms
- ▶ Even if all risk factors are known, not possible to always predict who will commit suicide
 - ▶ Use good clinical judgment,
 - ▶ follow closely and
 - ▶ order effective treatments.

Key Points Regarding Suicide (continued)

165

- ▶ Major suicide risk factors include:
 - Prior attempt
 - Family history of suicide
 - Presence of a psychiatric disorder
 - Hopelessness, impulsivity,
 - Alcohol or another substance use disorder

Key Points Regarding Suicide (continued)

166

- ▶ Strongest single factor for suicide is a prior suicide effort.
- ▶ Anyone who conveys a sense of hopelessness must be questioned about suicide.
- ▶ No one specific risk factor can be used to definitively predict suicide. Thus one must be knowledgeable of them **all** to do an adequate assessment.

Key Points Regarding Suicide (continued)

167

- ▶ Presence of an anxiety disorder in combination with depression doubles risk for suicide
- ▶ All suicide threats must be taken seriously and lead to a complete suicide risk assessment.



The Art of Warner Bros. Animation

By Steve Schneider

Foreword by Ray Bradbury